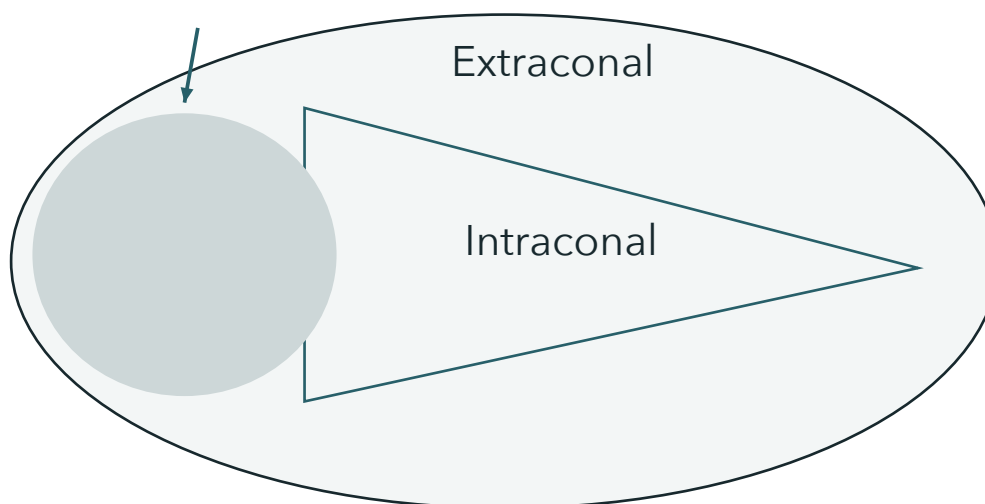


Orbital masses biopsy observation and surgery

Part 2 | Describe the compartment and displacement

An orbital mass needs more than a differential list.

Lacrimal fossa



Original compartment sketch: intraconal, extraconal and lacrimal-fossa relationships.

Record pain, growth, globe displacement, motility and optic function. Imaging should define compartment, tissue relationships and bone response before a route to tissue is chosen. A biopsy that is safe for one lesion may be inappropriate for another. In an oral case, state what the scan resolves and which finding would materially change the procedure.

Typical pleomorphic adenoma

Intact-capsule excision differs from an indiscriminate biopsy rule.

Finding	Implication
Classic presentation	Slow painless lacrimal-fossa mass
Typical imaging	Well-defined lesion with smooth remodelling
Operative aim	Remove intact without capsular violation
Atypical painful destructive lesion	Reassess diagnosis and tissue strategy

For a convincingly typical pleomorphic adenoma, deliberate capsule preservation guides the operative plan. Do not extend that principle to every lacrimal swelling or refuse tissue diagnosis for an atypical lesion. The useful answer explains why phenotype and imaging alter the recurrence risk and the route to diagnosis, while acknowledging uncertainty before surgery.

Suspected lymphoma: diagnosis and systemic extent

A local mass can be one manifestation of systemic disease.

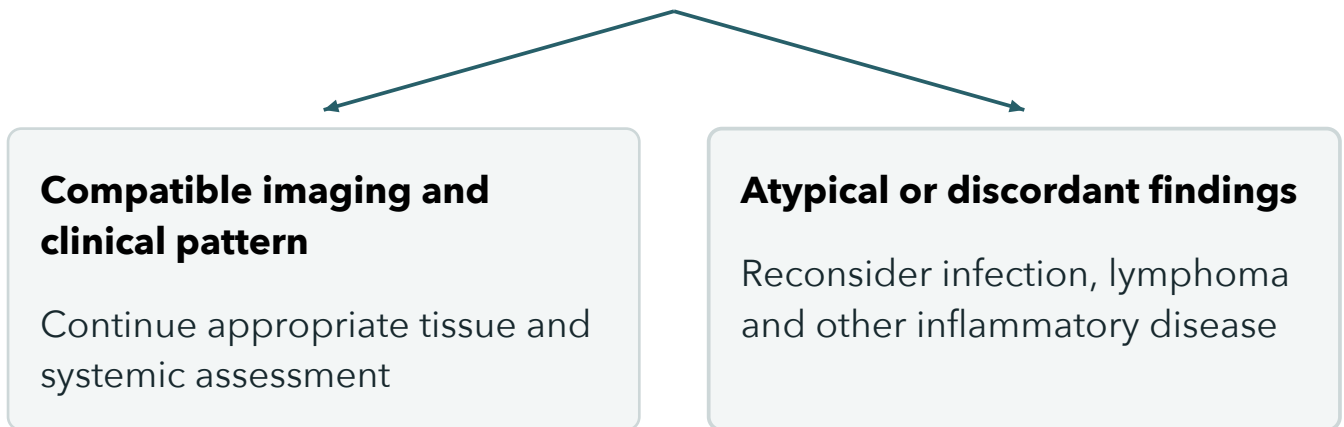
- Obtain adequate tissue using a planned safe approach
↓
- Request histology and appropriate ancillary studies
↓
- Coordinate systemic staging and haematology review
↓
- Select treatment for subtype and extent

A steroid response is not a substitute for a tissue diagnosis and may complicate subsequent interpretation. Avoid choosing a local definitive treatment before subtype and systemic extent are known. Explain the value of coordinated pathology, imaging and staging: each answers a different question and can change the balance between local and systemic therapy.

IgG4-related disease: serum is not definitive

Tissue and systemic context matter when the phenotype fits.

Can a normal serum IgG4 exclude orbital disease?

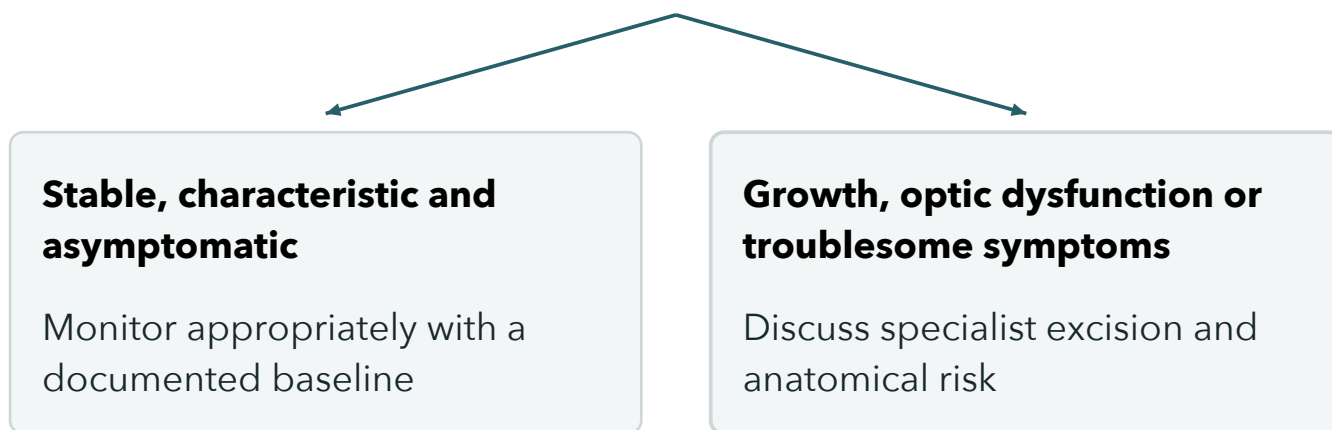


Bilateral lacrimal enlargement, muscle or infraorbital-nerve involvement may raise suspicion, but these findings are not unique. Interpret serum and histology with accepted clinicopathological criteria. A positive stain alone also requires context. State which competing diagnosis must be excluded before committing to prolonged immunosuppression and how systemic assessment affects ongoing care.

Cavernous venous malformation

An incidental stable lesion may be observed.

Is the lesion causing clinically important change?



The benign diagnosis does not dictate immediate surgery. Relate symptoms and longitudinal change to location, especially the optic nerve and orbital apex. Operative access and risk can vary substantially by compartment. Explain what would trigger reconsideration of observation, and avoid attributing unrelated symptoms to a radiological incidental finding without examination correlation.